

Your Caffeine Blueprint

Stop guessing when to stop - know your exact cutoff so caffeine works
for your focus, not against it

THE SCIENCE (30 SECONDS)

Caffeine has a half-life of 5-6 hours. Half the caffeine from your 2 PM coffee is still in your system at 8 PM. A quarter is still there at 2 AM.

You might fall asleep - but caffeine **reduces deep sleep by up to 20%**, even when you don't feel it.

After 2 weeks of daily use, caffeine stops giving energy - it only prevents withdrawal.

YOUR FORMULA

Take your target bedtime. Subtract 8-10 hours. That's your cutoff.

My target bedtime: _____ PM

Minus 8 hours = _____ (safe cutoff)

Minus 10 hours = _____ (ideal cutoff)

QUICK REFERENCE

BEDTIME	SAFE CUTOFF (-8H)	IDEAL CUTOFF (-10H)
10:00 PM	2:00 PM	12:00 PM (noon)
11:00 PM	3:00 PM	1:00 PM
12:00 AM	4:00 PM	2:00 PM
1:00 AM	5:00 PM	3:00 PM
2:00 AM	6:00 PM	4:00 PM

WHAT COUNTS AS CAFFEINE

SOURCE	CAFFEINE	EQUIVALENT
Espresso (1 shot)	63 mg	1 unit
Drip coffee (250 ml)	95 mg	1.5 units
Black tea (250 ml)	47 mg	0.75 units
Green tea (250 ml)	28 mg	0.5 units
Cola (330 ml)	32 mg	0.5 units
Energy drink (250 ml)	80 mg	1.25 units
Dark chocolate (50 g)	25 mg	0.4 units
Pre-workout supplement	150-300 mg	2-5 units

PRACTICAL TIPS

- **Time it, don't quit it.** Drink coffee in the morning - it works WITH your cortisol peak. After cutoff - decaf or herbal tea.
- **Hidden sources add up.** Dark chocolate, green tea, pre-workout. Check the table above.
- **Know your type.** ~15% are slow metabolizers (coffee at 3 PM = can't sleep). ~10% fast (coffee before bed, no problem). Rest are average. This is genetic. If 8 hours isn't enough - try a 10-12h cutoff.
- **Full reset.** Reduce by 1 cup every 3 days for 2 weeks. After that, one cup hits like it used to.

Medical disclaimer: This guide is for educational purposes only and is not medical advice. The information here is not intended to diagnose, treat, cure, or prevent any disease or condition. Consult a qualified healthcare provider before changing your sleep, diet, supplements, caffeine intake, or routines, especially if you have a medical condition, are pregnant, or take medications. If you experience persistent insomnia, suspected sleep apnea, loud snoring with daytime sleepiness, or other concerning symptoms, see a sleep specialist or your physician. In an emergency, call 911 or your local emergency services.

This guide is part of The ZeroFog. Based on caffeine pharmacokinetics research and clinical sleep medicine.